ADULT EMERGENCY LAPAROTOMY ANALGESIA GUIDELINE

STEP 1: SELECT A REGIONAL TECHNIQUE

FIRST LINE: Intrathecal Diamorphine & Rectus Sheath Catheters

- **Intrathecal Diamorphine 500-1000mcg. Effective for 18-24 hours.**
- **Rectus Sheath Catheters**
  - **Refer to local guideline**
  - **Catheters can remain in-situ for up to 72 hours.**

SECOND LINE: Rectus Sheath Catheters

- **For use when neuroaxial block is contraindicated***
- **Consider Additional Regional anaesthetic technique eg. Bilateral Erector Spinae Blocks (T8 Level)**

CONSIDER: Epidural (mid-thoracic)

- **Epidural analgesia consistently provides superior analgesia compared to other techniques**
- **HDU care is required.**

***NEUROAXIAL CONTRAINDICATIONS**

Absolute contraindications are:

- Local infection at insertion site
- Patient Refusal
- Existing or likelihood of septic shock (SBP <90, SBP <40% baseline or MAP <60), or Temperature >38oC
- INR ≥1.5
- No HDU/ICU bed available

Note: A raised White Cell Count or antibiotics does not necessarily preclude an epidural.

STEP 2: SELECT OPIOID SPARING SYSTEMIC ANALGESIA

A) Intra-operative

Consider opioid sparing option including:

- **Magnesium**
  50mg/kg up to 5g (20mmol) via slow iv administration. Monitor depth of NMB
- **Clonidine**
  0.5-1mcg/kg slow bolus. Avoid in hypotensive patients
- **Ketamine**
  Bolus 0.5mg/kg prior to incision. Caution in patients with IHD or arrhythmias.
- **Lidocaine**
  Bolus 1.5mg/kg over 10 minutes up to a max of 100mg prior to incision.
  Do not use alongside existing LA infusion such as epidural.

B) Post-operative

Consider short & long-term analgesia including:

- **Patient Controlled Analgesia**
  Morphine (recommended first line)
  Fentanyl 500mcg in 50ml with 10mcg bolus. (Contraindication to morphine)
  Regular Tramadol can be prescribed alongside a PCA. (Avoid with SSRIs)
  Prescribe regular **and** PRN antiemetic
- **Regular Paracetamol and an NSAID (if no contraindication) for every patient**